Custody Medical Transfer and Clinic Security Policy

Policy ID CRO-CMT-01 | Effective/as-of 1 February 2025 | Custody Operations

APPLIES TO	Escorted off-site diagnostic or medical transfers from custody
SOURCE STATUS	Operational policy; neutral configuration framework
MODAL KEY	shall / must = binding within this policy; should / may consider = supervisory recommendation

This policy uses a mandatory floor and a discretionary enhancement path. The configuration descriptions below are planning options; this policy does not select a configuration for a particular detainee.

SECTION 1 | Purpose and scope

P1.1 This policy applies to an escorted off-site diagnostic or medical transfer from custody. It establishes custody-transfer minimums, planning configurations, and clinic-security controls for the movement.

P1.2 This policy does not itself expand statutory or common-law authority. External law, medical judgment, and local operational facts remain controlling and must not be inferred from this policy.

SECTION 2 | Modal vocabulary

P2.1 Within this policy, shall and must state binding requirements. Should and may consider state recommendations that require responsible-supervisor judgment; recommended language is not a binding high-risk classification.

SECTION 3 | Mandatory baseline for an ordinary transfer

P3.1 For an ordinary off-site custodial medical transfer, the responsible supervisor shall approve the movement and identify the transport team before departure.

P3.2 The mandatory minimum transport team shall comprise: (a) one transport driver, who operates the vehicle and maintains team communications; and (b) one custody escort, who maintains direct custody control, conducts the identity and restraint check, and leads the controlled movement and handoff.

P3.3 Before departure, the driver and custody escort must complete and communicate identity, restraint, route, and communications checks. The policy sets no fixed departure lead time.

P3.4 At the receiving clinic, the custody escort shall retain custody control until a controlled handoff is completed with the receiving-site contact at a designated handoff point or through an equivalent controlled-access measure. The detainee shall not be left unattended in an uncontrolled public area.

P3.5 The receiving clinic controls clinical care. The custody and clinic-security roles in this policy do not diagnose, direct treatment, or substitute for external medical judgment.

SECTION 4 | Current high-risk triggers and supervisory judgment

P4.1 A current high-risk trigger exists when one or more of the following is present: (a) a current credible escape plan or escape attempt; (b) weapon access; or (c) active medical instability requiring continuous clinical monitoring.

P4.2 Documented past violence, including a violent offense or assaults while detained, is a significant factor for enhanced consideration. Past violence alone does not automatically convert recommended coverage into mandatory high-risk staffing when no current trigger in P4.1 is present.

P4.3 When a current high-risk trigger is present, the responsible supervisor shall use the enhanced mobile escort configuration described in P5.2.

P4.4 When a current high-risk trigger is present and the receiving clinic cannot ensure controlled access, a dedicated clinic-security post or equivalent controlled-access measure shall be used.

P4.5 When no current high-risk trigger is present, the responsible supervisor should consider enhanced mobile escort where history, behavior, transfer complexity, or the environment indicates a need for additional custody resilience. A dedicated

clinic-security post or equivalent controlled-access measure should be considered for a public-facing clinic without a dedicated secure holding room. These measures are recommended, not mandatory, on those facts alone.

SECTION 5 | Available planning configurations

P5.1 Configuration A - mandatory minimum escort with ordinary controlled clinic handoff. Staffing: one transport driver and one custody escort. Custody control: the custody escort maintains direct control through the controlled handoff. Clinic exposure: no dedicated police clinic post; access relies on the receiving-site arrangement and the controlled handoff. Operational burden: lowest, and this is the mandatory minimum for an ordinary transfer.

P5.2 Configuration B - enhanced mobile escort. Staffing: the Configuration A team plus one additional custody escort who travels with the team. Custody control: adds mobile observation and response coverage while the custody escort remains the direct controller. Clinic exposure: the additional escort is mobile, not a dedicated clinic-security post; public-facing clinic exposure remains unless the receiving site provides controlled access. Operational burden: additional personnel and coordination. Use is mandatory under P4.3 when a current high-risk trigger is present; otherwise it is a recommended enhancement under P4.5.

P5.3 Configuration C - coordinated enhanced arrangement. Staffing: Configuration B plus one dedicated clinic-security officer/post or equivalent controlled-access measure. Custody control: combines mobile resilience with an access-control and clinic-interface function; the custody escort remains responsible for direct custody. Clinic exposure: the post or equivalent measure addresses controlled access at the public-facing clinic and the absence of a dedicated secure holding room. Operational burden: highest of the three because it adds a fixed clinic function and coordination. The clinic-security function is mandatory under P4.4 when a current high-risk trigger is present and the clinic cannot ensure controlled access; under P4.5 it is recommended, not mandatory, when no current trigger is present.

P5.4 A clinic-security officer/post is separate from the additional mobile custody escort. Neither role performs clinical care. The policy does not establish staffing availability, cost, or a specific clinic layout.

SECTION 6 | Receiving-site dependency

P6.1 Before movement, the responsible supervisor or delegate should confirm the receiving-site contact, designated handoff point, and the available clinic-security post or equivalent controlled-access measure when Configuration C is selected. If the dependency cannot be confirmed, the supervisor must resolve the operational choice before departure; this policy does not prescribe a substitute clinic layout.

SECTION 7 | Separate emergency: medical collapse during an armed escape attempt

P7.1 A rare combined event in which a medical collapse occurs during an armed escape attempt is treated as a potentially fatal emergency. It is separate from ordinary transfer planning.

P7.2 On recognition, personnel shall immediately interrupt routine transfer activity and clearly communicate both threats: the medical emergency and the armed escape.

P7.3 Personnel shall seek emergency medical response, take life-safety action consistent with law and training, contain the scene, and notify the responsible supervisor or command channel.

P7.4 Ordinary custody movement shall not resume until the responsible supervisor or command channel provides the policy-required clearance after the emergency and threat have been addressed.

P7.5 An ordinary medical collapse, violent history, or routine transfer alone does not trigger P7.1 and does not make this exception the normal transfer staffing rule. P7.1-P7.4 do not independently authorize a search, use of force, diagnosis, or any other legal power.

SECTION 8 | Source limitations

P8.1 For this review, the policy is the supplied policy authority. It does not establish external legal powers, medical diagnoses, costs, staffing availability, or facts about a receiving site beyond what is expressly confirmed. Those matters require the applicable external authority, clinical judgment, or local operational confirmation.

Controlled document note: This fictional policy is frozen for the supplied review and is the sole policy authority for that review.